Ysbyty Gwynedd
Anaesthetics Department
Adran Anaestheteg

ACUTE PAIN

Intermittent intravenous (IV), intramuscular (IM) and subcutaneous (SC) Opioid Administration in Adults

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Key Points

- Intermittent IV / IM / SC opioids boluses are used for the management of moderate to severe pain associated with surgery, trauma or medical conditions or for the management of anticipated pain due to procedures.
- In many situations an oral route may be available and if appropriate should be used in preference.
- Intermittent IV opioids administration is reserved for the rapid titration of severe pain when the delay in absorption for an IM /SC route makes these routes inappropriate. **IV boluses should not be used for ongoing pain control.**
- Specific advice can be obtained from the Acute Pain service (APS) Monday – Friday (office hours) via pager 115. Out of Hours, contact On Call Anaesthetic team (bleep 063).

Exclusions:

This policy does not relate to opioids infusions, Patient Controlled Analgesia (PCA`s), Nurse Controlled Analgesia (NCA`s) or to opioids administration within the operating theatre and theatre recovery. It does not apply to patients in whom an “end of life pathway” document is in place. In addition there may be some patients under the care of the different specialist pain teams where some aspects of this guideline may need to be modified. Advice on these patients will be given by the individual specialist pain team.

Contraindications:

- See exclusions above
- Known significant allergic reaction to opioids.
- Patients with significant respiratory depression or likely airway compromise.
- Clinical areas where there is no ability to monitor the patient appropriately (see below).

Prescription:

- The opioids to be administered should be prescribed on the treatment sheet as per BCU policy including information on the dose, route and frequency of administration.
- A suitable prescription for naloxone should be written.
- In most situations morphine is the most commonly used IV opioid. This should be prescribed and administered in most circumstances as no more frequently than 1mg to 2mg every 5 minutes.
- A consideration should be given to a complementary prescription of simple analgesics.
- Consideration should be given for an appropriate prescription to provide any ongoing analgesic requirements.

Precautions:

If the patient is receiving other medication that may contribute to sedation then smaller doses of opioids may be required and the staff should be alert to the increased risk of sedation and / or respiratory depression.

Designated areas of Responsibility:

- The prescriber must ensure the prescription is safe for the individual patient taking into account their clinical condition, concomitant drugs and allergies.
- The person administering the drug should ensure the clinical condition of the patient necessitates the drug.
- All Legal requirements and Health Board policies relating to prescription administration, storage, record keeping and disposal of controlled drugs must be followed.
- Prior to and following opioid administration the patient should be monitored as follows:
    1. Respiratory rate, sedation score, pain score, pulse, blood pressure. These should all be recorded **before administration** and then following administration every **30 minutes for 1 hour** and then **4 hourly** unless the clinical condition dictates more frequently.
    2. In the case of IV administration as well as the above applying, the patients respiratory rate, sedation score, pain score, blood pressure and pulse should be recorded between each bolus and after the last bolus has been given. Two further sets of observations should be undertaken at 5 minute intervals. The pattern outlined in 1 above should then be followed.
- If prior to administration the patient has a respiratory rate of less than 10 per minute or is over sedated then the opioid should not be administered.
- Pharmacy is responsible for ensuring a stock supply of naloxone is available in those clinical areas where this is deemed to be a stock item.

Management of Side Effects:

- In the event of respiratory depression(less than 10 / min) no further opioid should be administered.
- If the respiratory rate drops to **less than 8/ min** give oxygen, naloxone should be prepared (naloxone 0.4mg & 3mls saline) and administered IV at 1-2ml increments until the respiratory rate is more than 10/ min and the patient is not over sedated. A “Medical Emergency” call may be required.
- In the event of respiratory arrest a “Cardiac Arrest” call should be made and naloxone administered as above.
- Other side effects of opioids should be brought to the attention of the clinical staff and treated as appropriate.
- In the event of any adverse event the BCU policy on this should be followed. A “yellow card” may need to be completed as may a BCU Datix form.

References

- British Medical Association, Royal Pharmaceutical Society of Great Britain: British National Formulary September 2021- March 2022 (82).
- BCUHB Medicines Code – (current version)